Heparin (to prevent occlusion of arterial lines)

Caution: Special care required, has been confused with other continuous infusions in neonates. Different strengths available.

FORM

Heparin sodium 1000IU/L in 0.45% w/v sodium chloride IV infusion (premade bag, 500ml)

and

Heparin sodium flushing solution 50 units in 5ml vial - **Only use if IV infusion bag not available**

INDICATION

To prevent occlusion of arterial lines.

DOSE RANGE

AGE	DOSE	FREQUENCY	ROUTE
Neonate	0.5ml/hour – 1ml/hour*	Continuous infusion	Continuous Intra-arterial infusion

*1ml/hr is accepted practice in some larger infants where occlusion still occurs @ 0.5ml/hr. Caution as this rate increases heparin delivery

RECONSTITUTION

Not required when infusion bags available

DILUTION

Heparin sodium 1000IU/L in 0.45% w/v sodium chloride IV infusion bag 500ml – no dilution required. Directly withdraw 50ml in to syringe

If IV infusion bags are not available
Heparin sodium flushing solution 50 units in 5ml vial

Heparin 50units in 5ml	5ml
Sodium chloride 0.45%	to 50ml

Gives 1unit per ml solution.

METHOD OF ADMINISTRATION

Continuous infusion

CAUTIONS, CONTRA-INDICATIONS AND SIDE EFFECTS
- See Summary of Product Characteristics and most recent edition of BNF for Children

SODIUM CONTENT

Heparin sodium 1000IU/L in 0.45% w/v sodium chloride IV infusion 500ml: **0.11mmol/ml**

Heparin sodium flushing solution 50 units in 5ml vial diluted as detailed above: **0.08mmol/ml**

PH

5-7.5

Neonatal

NEONATAL West of Scotland Drug Monographs
Parenteral Route

LICENSED STATUS Licensed special

APPLICABLE POLICIES West of Scotland Neonatal Guidelines

Consult local policy if applicable

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Administer reconstituted solutions immediately.
All vials, ampoules and infusion bags are for single use only unless otherwise stated.

Dose may vary depending on indication, age, renal function, hepatic function, and concomitant medications.
This monograph should be used in conjunction with the terms of reference document prepared by the West of Scotland Pharmacist Group. Information is correct at the time of publication and as per local practice agreement.
For further advice please contact your clinical pharmacist or pharmacy department